A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

17 April 2026

Dr Sung-Hee Moon
Eltham Ridge Medical Centre

Dear Dr Moon,

Re: Mr William Li
DOB: 1981-04-12
Address: 2/1 Hill Court, MACLEOD, VIC, 3085

Summary

- **Ulcerative colitis in clinical and biochemical remission. on Subcutaneous Vedolizumab**
- **Minor episode of rectal bleeding not suggestive of a clinical flare.**
- **Surveillance colonoscopy scheduled for 2027.**
- **Routine review in six months with interval pathology.**
- **No action required by you.**

I had the pleasure of conducting a review with William Li regarding the management of his ulcerative colitis. William remains largely asymptomatic and is doing well. He did report a single episode of minor rectal bleeding limited to wiping; however, as this was not mixed with the faeces and resolved the following day, I do not believe this represents a significant flare of his condition.

William's recent investigations are very reassuring. His blood results are unremarkable, and a faecal calprotectin level from April was less than 50 mg/kg, confirming biochemical remission. We discussed that as he has recently turned 45, he may receive correspondence regarding the National Bowel Cancer Screening Program. I have advised him that he does not need to participate in this programme, as he is already under a formal specialist endoscopic surveillance schedule. While his inflammation is limited to the distal bowel, I believe it is appropriate to maintain regular monitoring.

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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Impression and Plan William is currently in stable remission. I have planned for his next surveillance colonoscopy to take place in 2027, which maintains a three-year interval from his previous procedure. I have provided him with pathology referrals for bloods and a repeat faecal calprotectin to be completed in six months. I will review him again in October. If he experiences an escalation in symptoms or if his inflammatory markers rise, I will consider advancing the date of his colonoscopy.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: Ms Helen Aharfi

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